

إِسْم الْمَرِيضِ رِبَاعِيًّا : عُبَيْرُ مُحَمَّدٍ مُحَمَّدُ زَكِي

الحي / المدينة : النزهه

المحافظة : القاهرة

رقم البطاقة : 26906280100428 رقم التلفون : 01273701035

العمر(تاريخ الميلاد) : 28-06-1969 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : لوكيميا

نتائج الأبحاث : (مرفق)

التشخيص النهائي : لوكيميا

خطة العلاج : أورام كيمائي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا **قسم العلاج :** علاج هرموني حمض الزيلودرونيك [14157] امراض الدم

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.

كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جہۃ العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

Name: عبيد محمد محمد زكي
National ID: 26906280100428



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد أنه

المريض يعاني من لوكيميا ليفماو به حاده ويحتاج الى Zoledronic acid for bone protection during hormonal therapy

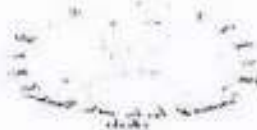
الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



MDT form

A-Patient ID:

1- Name : **تيمور محمد زكي**

Sex:

3- Medical number:

Female

83155285

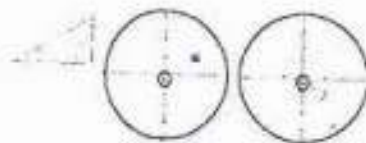
Old Med :

Do not:

5574-2020

2-ID number:

Patient Age : 56 Y



Family history

Family history

Phone

Home

01273701035

BMI

DH

REASON OF MDT

REASON OF MDT
+ REMIT

Marital status

Marital status

NRS Score

NRS Score

GROUP

SADUP

Date of MDT

Date of MDT

Co morbidity

Co morbidity

Menopausal
status

Menopausal status
REASON OF MDT

Initial STAGE

Initial STAGE

05/01/20

Clinical data

Breast Imaging :

Mets Workup :

CT CAP& bone scan:multiple bone+mediastinal+ LNs

BX IDC	ER 7/8	PR 5/8	Her 2 Neu 3	SISH	Ki67
BX COMMENT	ER COMMENT	PR COMMENT	Her 2 Neu COMMENT	SISH COMMENT	

T	N	M 1 MEDIASTINA
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Sonomammography

PATIENT HISTORY

RT MRM PT2N3(12/2020)

ADJ acX4 + TAXOL X 12 + HERCEPTIN 1 YR ENDED 9/2022

on adj AI

CT CHEST:MEDIASTINAL LN

pt finished last session taxol/herceptin 29/1/2024

---> DP shifted to tykerb / aromasine

ct cap: size progression of enlarged mediastinal , hilar LN

mild progression of multiple predermal lytic osseous lesion

last mdt:fasoldex+zometa ---> refused by MOH

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

for xeloda single agent + monthly zometa

SIGNATURE			
MEDICAL ONCOLOGY	MEDICAL ONCOLOGY 10/1/2024	RADIOLOGY	RADIOLOGY 10/1/2024
SURGERY	SURGERY 10/1/2024	PATHOLOGY	PATHOLOGY 10/1/2024
Psychiatric	Psychiatric	Nutrition	Nutrition

Name: عبيد محمد محمد زكي		
MR. : 126		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه MBC TO BONE FOR XELODA + ZOMETA XELODA 3X4X14 NECK US FREE - NON SPECIFIC LN postm mbc to bone med ln idc er7/8 pr5/8 her23+ .history rt m,rm 2020 adj chemo herceptin port ai till 10/2023 mets to bone med ln st taxol herceptin dp in 4/2024 st aromasin tykerb that was stopped bec of toxicity till12/2024 dp st xeloda from1/2025 cts 11/2025 sd bs1/2026 sd for new cts for xeloda 4 morning 3 evening for14 d for pall rth CT CAP12\4\2026 >> SD , BS >> R PT RECIEVED PALLIATIVE RTH TO LT SHOULDER & LT HIP JOINT 6\4\2026 XELODA 4\3 TABS Pt came today on wheelchair Ps III e poor oral intake Bone scan 31-1-2026 >>> SD today for xeloda 4am 3 Pm *14 Pt received Tykerb for 2 mns and cannot tolerate it since 2024 labs >>> OK for nutritional cons after Dr hanan cons >> for re-challeng with tykerb with dose reduction , supp tt for tykerb >> r MBC TO BONE FOR ZOMETA TODAY HB 6.5 (FOR 2 PACKED RBCS TODAY LAST WEEK HB 6.5 > RECEIVED 2 PRBCs, for new CBC then for xeloda/tykerb + zometa if labs ok A CASE OF BREAST CANCER METASTATIC TO BONE ON TYKERB XELODA TYKERB APPROVAL STILL PENDING CBC SHOWING ANEMIA HB 6.5 NB PT HAS DVT 10 DAYS AGO AND STARTED CLEXAN 60MG SC SO TODAY FOR BLOOD TRANSFUSION THEN RECEIVE XELODA CT without compariosn, showing subpleural pulmonary nodules>> for comparison with prev CT + mixed lytic and sclerotic bony lesions, also for comparison to determine disease course meanwhile for xeloda + zometa (ca level is elevated showing hypercalcemia total ca 14.5, for ionsided ca + zometa + iv fluids HB 6.7 FOR 2 RBCs TRANSFUSION + RE-CBC FOR ER FOR NEW CT CAP WITH COMPARISON + MRI BRAIN nb: there's an error in registering the patients imaging requests in the system hb :7 for blood transfusion xeloda 3*2*14 waiting tykerb approval metasic breast cancer to the bone will increase tramadol to 50 mg / 6h paracetamol 500 mg/ 6h		
تحريرا في ,		
	التاريخ :	19-09-2026
	الوقت :	
	هنا أسامه عبد الرحيم عبد البر	الطبيب المعالج :
يعتمد ,		



مدير المصالح
بمركز المصالح
قسم الباثولوجي

Pathology Report

Name: عبيد محمد نكي

Age: 50 years.

Clinical data: Right breast mass.

Specimen type: Right MRM

Date(received) : 26/12/2020

Date(reported) : 4 /1/2021

Lab. Number: 2157

Hospital Number: 5574 /2020 نفقة دولية

Gross Description:

Right MRM specimen : measured 20x16x6cm, covering skin 20x14cm, axillary fat 8x4cm, nipple was unremarkable , cut section revealed UOQ ill-defined firm greyish white mass 2.5x2x1cm, away 3cm from skin and 2cm from deep margin, axillary fat dissection revealed 20 nodules up to 3.5cm.

Microscopic Examination

Sections examined revealed breast tissue infiltrated by invasive , malignant tumor formed of nests and cords of moderately anaplastic ductal epithelial cells with desmoplastic stroma.

The examined axillary LNs showed heavy metastatic deposits in 18 nodes (18/20) with extra capsular extension.

Diagnosis:

Right breast mass, Right MRM.

Invasive duct carcinoma grade II.

Minimal DCIS .

Positive lymphatic tumor emboli.

Negative perineural invasion.

Free skin, nipple & margins .

L.Ns status(18/20) with extra capsular extension.

Stage p T2 p N3a.

Prof. Gamal Aba L. Aty.
Dr. Enas Mohammed Ali

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 74/2025	Pat. ID: 20109

Patient Name: Abeer Mohamed Mohamed Zaky

Date: 29-7-2025

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

HISTORY : A known case of breast cancer underwent right mastectomy .As compared to the previous report dated 17/04/2025,the current study reveals:

FINDINGS:

- Evidence of right mastectomy with clear operative bed
- Still noted stable middle lung lobe subpleural reticulations mostly representing post irradiation sequel .
- Still noted bilateral basal & middle subpleural atelectic bands are seen
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses or consolidative patches
- Stationary course as regards prevascular heterogenous nodal lesion with internal tiny foci of calcifications measuring about 4.7 X 2.9 cm.
- Still noted enlarged mediastinal and bilateral hilar lymph nodes ,the largest is subcarinal location measruing about 2.4 x 3.7 cm as compared to in the previous study 2.6 x 4.4 cm.
- Normal C.T. appearance of the mediastinum vessels.
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- Still noted multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton with lytic burden more at the manubrium sternal lesion.

CONCLUSION :-

Follow up study with overall stationary course of the disease.



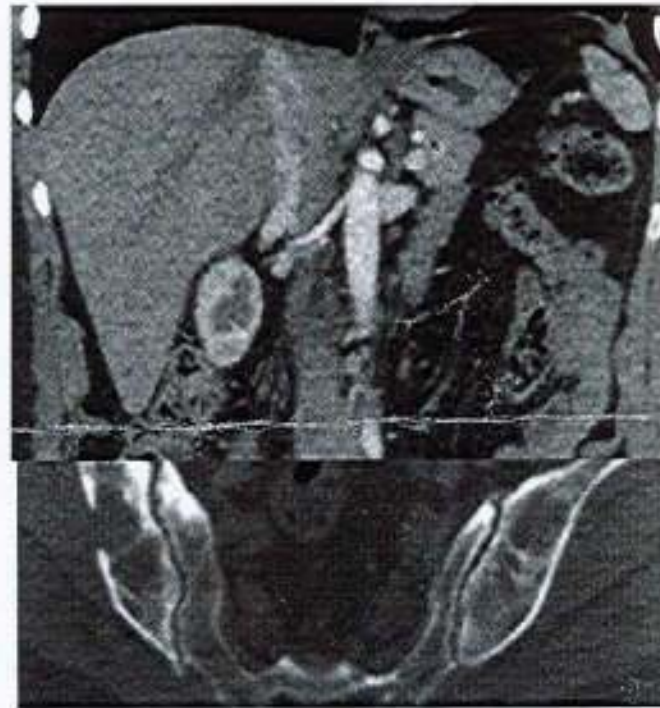
Patient Name: Abeer Mohamed Mohamed Zaky	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 74/2025	Pat. ID: 20109

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS-:

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

- Normal C.T. density of the liver parenchyma, with no focal lesions .
- No extra or intra-hepatic biliary dilatation .
- Both kidneys are functioning and normal in dimensions.
- The spleen and pancreas show normal C.T. appearance .
- Normal size and C.T. density of the adrenal glands .
- No ascites .
- The urinary bladder shows normal size and wall thickness with no calculi inside.
- The uterus is of normal C.T. appearance .
- Normal ischio-rectal fossae on both sides .
- No sizable abdominal or pelvic lymph glands.
- **Still noted** multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton .



IMPRESSION:

Overall stationary course of the disease

Much Obligated,

Mona Ali,MD

Not Approved

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 15/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 56 Year(s)
Medical ID: 5574/2020	Pat. ID: 20109

POST CONTRAST C.T. SCAN OF THE CHEST:-

TECHNIQUE: Axial contiguous thin cuts were taken through the chest after I.V. contrast injection.

HISTORY: A known case of breast cancer underwent right mastectomy .As compared to the previous report dated 26/7/2025, the current study reveals:

FINDINGS:

- Evidence of right mastectomy with clear operative bed
- Still noted stable middle lung lobe subpleural reticulations mostly representing post irradiation sequel .
- Still noted bilateral basal & middle subpleural atelectic bands are seen
- The rest of both lungs show normal appearance of its parenchyma and pulmonary vasculature with no evidence of abnormal masses or consolidative patches
- Stationary course as regards prevascular heterogenous nodal lesion with internal tiny foci of calcifications measuring about 3.4x4.4 cm was 4.7 X 2.9 cm ?granulomatous.
- No evidence of sizable mediastinal or hilar lymph nodes.
- Normal C.T. appearance of the mediastinum vessels.
- Patent tracheo bronchial tree.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- Still noted multiple lytic and sclerotic bony lesions involving the axial and appendicular skeleton with lytic burden more at the manubrium sterni lesion.



CONCLUSION :-

Follow up study with overall stationary course of the disease as regards previous report.

POST CONTRAST C.T. SCAN OF THE ABDOMEN AND PELVIS:-

TECHNIQUE: Axial contiguous thin cuts were taken through the abdomen and pelvis after I.V. contrast injection.

FINDINGS:

- Normal C.T. density of the liver parenchyma, with no focal lesions .

Patient Name: Abeer Mohamed Mohamed Zaky	Date : 17/08/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 56 Year(s)
Medical ID: 5574/2020	Pat. ID: 20109

MRI OF THE BRAIN WITH CONTRAST

Findings:

- No cerebral parenchymal areas of abnormal signal intensity.
- no evidence of enhancing cerebral or meningeal lesions.
- Normal size and configuration of the ventricular system with no evidence of deformity or displacement.
- No cerebellum or brain stem areas of abnormal signal intensity
- No mid line shift.
- Examined PNS are unremarkable .

OPINION:

known case of breast cancer:

- No evidence of metastasis. .
- No recent ischemic insult.

Dr. Reham Khedr (MD)

Signature Reeham.Khader
19/08/2025 7:24 PM



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 23/04/2024
Exam : C.T CERVICAL SPINE	Age : 55 Year(s)
Medical ID: 5574/2020	20109

MRINON CONTRAST CT OF THE WHOLE SPINE

FINDINGS:

- Mild progressive course of the right iliac bone lytic osseous lesions.
- Otherwise stationary other bilateral pelvic bones and few vertebral predominately lytic osseous lesions
- Normal dimensions of the spinal canal.
- No evidence of vertebral collapse.
- Normal size of the conus medullaris.
- No para-spinal soft tissue masses or collection.

OPINION:

- Mild progressive course of the right iliac bone lytic osseous lesions.
- Stationary rest of the study.

Dr. Ali Saber, MSc.

Signature Prof. Menna Tallah Hanafy

30/04/2024 12:16 م



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 09/02/2023
Exam : MAMMOGRAPHY BOTH	Age : 53 Year(s)
Medical ID: 5574/2020	

LEFT BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

Patient had right MRM

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- fatty parenchyma of the left breast (ACR a)
- No speculated masses or suspicious micro calcific clusters of the left breast.
- Complementary ultrasound showed no left solid or cystic mass lesions.
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts of the left breast.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions.
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- Essential normal study of the left breast (BIRADS 1)

Best regards,
Dr/Asmaa Zeinhom, Msc

Signature Temp
09/02/2023 2:03 PM



Patient Name: Abeer Mohamed Mohamed Zaky	Date : 23/10/2024
Exam : Item 1	Age : 55 Year(s)
Medical ID: MOSTAFA^BRAIN	Pat. ID: 20109

MRI STUDY OF THE BRAIN

Technique of Examination:

- Axial T1, T2 & FLAIR WIs
- Sagittal T1WIs
- Post contrast series

MR Findings:

- No definite enhancing intra cranial masses.
- No cerebral parenchymal areas of abnormal signal, yet a left cerebellar small subcentimeteric lacunar infarct noted (displaying fluid signal) with no surrounding mass effect.
- Normal grey / white matter differentiation.
- Normal size and configuration of the ventricular system.
- No shift of midline structures.
- No areas of fresh blood or blood degradation product signal.
- Normal appearance of the brain stem and cerebellum.
- No extra axial collections.

Opinion:

- No definite enhancing intra cranial masses.
- A left cerebellar small old lacunar infarct.

Dr. Sherif Sameh, M.D.

Health Insurance Organization
Nasr City Cancer Center
(NCCC)



شركة التأمين الصحي
مركز أور او مدينة نصر

اسم المريض : حبيب محمد محمد

تاريخ الخدمة : 2020/10/27

Pathology Report

Lab.No:20/811

Specimen:

Right breast mass.

Gross:

six cores, each 1.2 cm long, totally submitted.

Diagnosis:

Right breast mass, tru cut biopsy:

-Invasive duct carcinoma grade II

With DCIS (cribriform, solid pattern in 30% of limited biopsy)

Prof.Dr.Hussain Elsayed

Doc: 11/11/2020 Time: 14:14:14



DMS
مدى



جمهورية مصر العربية
بطاقة تحقيق الشخصية

عبر
محمد محمد زكي
٢٦ ش احمد فهد بيومي مصر الجديدة
النزهة - القاهرة

١٩٦٩/٠٦/٢٨ ٢٦٩.٦٢٨.٠١.٠٠٤٢٨
KC2789262

٢٠٢٢/٠٨ ٢٦٩.٦٢٨.٠١.٠٠٤٢٨
ربة منزل
أنثى
متزوجة
مسلمة
باتقا الفكي محمد باتقا
البطاقة سارية حتى ٢٠٢٠/٠٨/٠١

١

9-17

د. يوسف روستا

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

المنشأة المصرية

MDT FORM

09/16/2026

Name:	Abir Mohammad Zaky Mohammad	MRN:	126
Age:	57 Y	ID:	26906280100428
Menopausal Sta		Phone:	01210939545
NRS SCORE:	ps 3	Marital Status:	married
BMI:		Family History:	
CO Morbidity:	-ve	Reason Of MDT:	mbc

Clinical Data:

BX:

ER: 7/8

PR: 5/8

HER2: +3

KI67:

Sonomammography:

Mets Workup:

2020 :rt mrm (t2 n3)
adjuvant : ac*4+taxol *12+herceptin 1 year (till 1/2024)
2024:tykerb +aromasin
1/2025 :single agent xeloda till now
8/2026 CT CAP :
1/Multiple hepatic, pulmonary and osseous metastatic lesions.
2/Progressive course since the last study dated 26-7-2026
8/2026 bone scan :
**Stationary widespread osseous metastases, showing no significant interval progression
8/2026 MRI BRAIN :
Bilateral cerebellar few small foci of CSF signal, likely old ischemic Partial foci
empty sella
Prominent extra axial CSF spaces (age related)
• No acute brain infarction
•No evidence of gross SOL or mass effect on non contrast base

History:

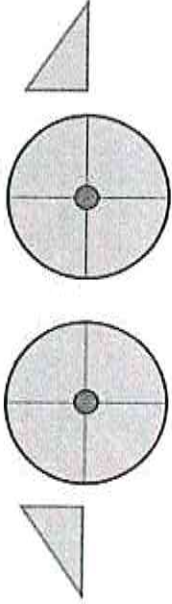
Recommendations:

MDT Outcome & Plan

For:
1.Re-do MRI brain with contrast
2.Start Taxol/Carbo weekly + Herceptin (if not approved by MOH, trial through NGO)

R

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. شيماء لاشين	د. امليه عيسوي				

16/9/2026
16/9/2026

Patient's Name:

Abber Mohammed Mohammed Zaki

Date:

2026-08-27

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED:

I- CHEST:

- Progressive course regarding the size and number of the multiple bilateral pulmonary nodules the largest is seen in the left apical segment of the right lower lobe measuring 17 x 15 mm.
- Newly developed Diffuse interstitial septal thickening is seen involving --- lungs associated with peribronchial thickening; likely suggestive of lymphangitis carcinomatosis.
- Still noted superior mediastinal soft tissue with calcifications ?? nodal measuring 40 x 33 x 30 mm
- Scattered multiple mediastinal lymph nodes.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
- Newly developed multiple hepatic hypodense focal lesions the largest is seen in the left lobe measuring 20 x 19 mm showed heterogenous mainly peripheral enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple lytic and sclerotic osseous lesions seen in bilateral iliac bones, dorsal and lumbar vertebral bodies as well as multiple ribs

Opinion:

- ❖ Multiple hepatic, pulmonary and osseous metastatic lesions.
- ❖ Progressive course since the last study dated 26-7-2025

Dr.Asser Ahmed

Patient's Name:

Abeer Mohammed Mohammed Zaki

Date:

2026-08-23

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Examination:

■ MRI of the brain without contrast

Technical data:

- Routine pulse sequences without contrast

Indication: dizziness, headache, blurred vision, history of operated breast cancer

Comparison: none

Findings:

- Bilateral cerebellar few small foci of CSF signal, no perifocal edema or related mass effect.
- Partial empty sella
- Prominent extra axial CSF spaces
- Normal signal of the cerebral hemispheres parenchyma with no focal mass lesion or areas of abnormal signals.
- Preserved gray white matter interface
- Normal size and shape of the ventricular system.
- No midline shift.
- No extra axial fluid collections.
- Normal appearance of the brainstem with no focal lesions or areas of abnormal signal.

Conclusion:

- Bilateral cerebellar few small foci of CSF signal, likely old ischemic foci
- Partial empty sella
- Prominent extra axial CSF spaces (age related)
- No acute brain infarction
- No evidence of gross SOL or mass effect on non contrast base

Recommendation:

- Clinical correlation and follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Name : Abner Mohammed Mohammed Zaki
Branch : Dokki
Ref Physician Prof : Goustay Roussy Egypt

Date : 31-07-2026 06:09
Medical No. : 306197
In Company : شركة مكنيل

Whole-Body Bone Scintigraphy Report

Clinical History: 57-year-old female patient with history of right breast carcinoma, status post-surgical resection, chemotherapy, and radiotherapy. Evaluated in comparison with the prior study dated January 31, 2026.

Findings:

- Widespread active osteoblastic activity is seen throughout the axial and appendicular skeleton, involving the skull, sternum, bilateral shoulders, scapulae, ribs, multi-level spine, pelvic bones, femora, and right humeral shaft.
- Comparison with the prior study dated January 31, 2026 demonstrates no significant interval change in the overall distribution, number, or intensity of these metastatic osseous lesions.
- Co-existing mild degenerative changes are noted along the spine and major weight-bearing joints.

Impression:

- Stationary widespread osseous metastases, showing no significant interval progression or regression compared to the prior study dated January 31, 2026.

Dr. Alaa Nagi,
MD Nuclear Medicine

Patient's Name: Abeer Mohammed Mohammed Zaki
Date: 2026-08-23
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

▪ MRI of the brain without contrast

Technical data:

- Routine pulse sequences without contrast

Indication: dizziness, headache, blurred vision, history of operated breast cancer

Comparison: none

Findings:

- Bilateral cerebellar few small foci of CSF signal, no perifocal edema or related mass effect.
- Partial empty sella
- Prominent extra axial CSF spaces
- Normal signal of the cerebral hemispheres parenchyma with no focal mass lesion or areas of abnormal signals.
- Preserved gray white matter interface
- Normal size and shape of the ventricular system.
- No midline shift.
- No extra axial fluid collections.
- Normal appearance of the brainstem with no focal lesions or areas of abnormal signal.

Conclusion:

- Bilateral cerebellar few small foci of CSF signal, likely old ischemic foci
- Partial empty sella
- Prominent extra axial CSF spaces (age related)
- No acute brain infarction
- No evidence of gross SOL or mass effect on non contrast base

Recommendation:

- Clinical correlation and follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Patient's Name: Abeer Mohammed Mohammed Zaki
Date: 2026-08-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MULTISCIJCE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED:

I- CHEST:

- Progressive course regarding the size and number of the multiple bilateral pulmonary nodules the largest is seen in the left apical segment of the right lower lobe measuring 17 x 15 mm.
- Newly developed Diffuse interstitial septal thickening is seen involving --- lungs associated with peribronchial thickening; likely suggestive of lymphangitis carcinomatosis.
- Still noted superior mediastinal soft tissue with calcifications ?? nodal measuring 40 x 33 x 30 mm
- Scattered multiple mediastinal lymph nodes.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles.
- Newly developed multiple hepatic hypodense focal lesions the largest is seen in the left lobe measuring 20 x 19 mm showed heterogeneous mainly peripheral enhancement
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple lytic and sclerotic osseous lesions seen in bilateral iliac bones, dorsal and lumbar vertebral bodies as well as multiple ribs

Opinion:

- ❖ Multiple hepatic, pulmonary and osseous metastatic lesions.
 - ❖ Progressive course since the last study dated 26-7-2025
- Dr.Asser Ahmed*

جوسافي
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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الهيئة المصرية
للتنظيم
الطبي

MDT FORM

09/16/2026

Name:	Abir Mohammad Zaky Mohammad	MRN:	126
Age:	57 Y	ID:	26906280100428
Menopausal Sta		Phone:	01210939545
NRS SCORE:	ps 3	Marital Status:	married
BMI:		Family History:	
CO Morbidity:	-ve	Reason Of MDT:	mbc

Clinical Data:

R

BX:

idc

ER: 7/8

PR: 5/8

HER2: +3

KI67:

Sonomammography:

Mets Workup:

2020 :rt mrm (t2 n3)
adjuvant : ac*4+taxol *12+herceptin 1 year (till 1/2024)
2024:tykerb +aromasin
1/2025 :single agent xeloda till now
8/2026 CT CAP :
1/Multiple hepatic, pulmonary and osseous metastatic lesions.
2/Progressive course since the last study dated 26-7-2026
8/2026 bone scan :
**Stationary widespread osseous metastases, showing no significant interval progression

8/2026 MRI BRAIN :
Bilateral cerebellar few small foci of CSF signal, likely old ischemic Partial foci
empty sella
Prominent extra axial CSF spaces (age related)
• No acute brain infarction
•No evidence of gross SOL or mass effect on non contrast base

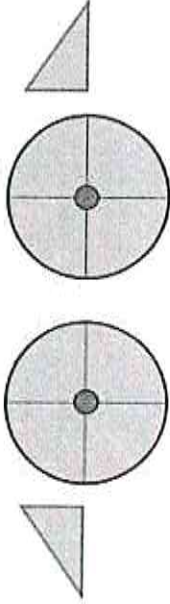
History:

Recommendations:

MDT Outcome & Plan

For:
1.Re-do MRI brain with contrast
2.Start Taxol/Carbo weekly + Herceptin (if not approved by MOH, trial through NGO)

L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د شيماء لاشين	د اشيه عيسوي				

١٦/٩/٢٠٢٦
Omar
16/9/2026

Patient's Name:

Abber Mohammed Mohammed Zaki

Date:

2026-08-27

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED:

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- No abdominal or pelvic lymphadenopathy.
- No ascites.
- Multiple lytic and sclerotic osseous lesions seen in bilateral iliac bones, dorsal and lumbar vertebral bodies as well as multiple ribs

Opinion:

- ❖ Multiple hepatic, pulmonary and osseous metastatic lesions.
- ❖ Progressive course since the last study dated 26-7-2025

Dr.Asser Ahmed

Patient's Name:

Abeer Mohammed Mohammed Zaki

Date:

2026-08-23

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

Examination:

■ MRI of the brain without contrast

Technical data:

- Routine pulse sequences without contrast

Indication: dizziness, headache, blurred vision, history of operated breast cancer

Comparison: none

Findings:

- Bilateral cerebellar few small foci of CSF signal, no perifocal edema or related mass effect.
- Partial empty sella
- Prominent extra axial CSF spaces
- Normal signal of the cerebral hemispheres parenchyma with no focal mass lesion or areas of abnormal signals.
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Conclusion:

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- No acute brain infarction
- No evidence of gross SOL or mass effect on non contrast base

Recommendation:

- Clinical correlation and follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Name : Abner Mohammed Mohammed Zaki
Branch : Dokki
Ref:Physician Prof : Goustay Roussy Egypt

Date : 31-07-2026 06:09
Medical No. : 306197
In: Company : شركة

Whole-Body Bone Scintigraphy Report

Clinical History: 57-year-old female patient with history of right breast carcinoma, status post-surgical resection, chemotherapy, and radiotherapy. Evaluated in comparison with the prior study dated January 31, 2026.

Findings:

- Widespread active osteoblastic activity is seen throughout the axial and appendicular skeleton, involving the skull, sternum, bilateral shoulders, scapulae, ribs, multi-level spine, pelvic bones, femora, and right humeral shaft.
- Comparison with the prior study dated January 31, 2026 demonstrates no significant interval change in the overall distribution, number, or intensity of these metastatic osseous lesions.
- Co-existing mild degenerative changes are noted along the spine and major weight-bearing joints.

Impression:

- Stationary widespread osseous metastases, showing no significant interval progression or regression compared to the prior study dated January 31, 2026.

Dr. Alaa Nagi,
MD Nuclear Medicine

Patient's Name: Abeer Mohammed Mohammed Zaki
Date: 2026-08-23
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

▪ MRI of the brain without contrast

Technical data:

- Routine pulse sequences without contrast

Indication: dizziness, headache, blurred vision, history of operated breast cancer

Comparison: none

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Recommendation:

- Clinical correlation and follow up

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Patient's Name: Abeer Mohammed Mohammed Zaki
Date: 2026-08-27
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

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Opinion:

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 - ❖ Progressive course since the last study dated 26-7-2025
- Dr.Asser Ahmed*